



Final round 2

3. VITAL SIGNS

Heart rate	80 – 100 bpm
Respiratory rate	12-20 rpm
Blood pressure	110-120/60 mmHg
Temperature	37 °C (98.6 °F)

4. HEMATOLOGY VALUES

RBCs	4.5 – 5.0 million per mm ³
WBCs	4,500 – 11,000 per mm ³
Neutrophils	60 – 70%
Lymphocytes	20 – 25%
Monocytes	3 – 8%
Eosinophils	2 – 4%
Basophils	0.5 – 1%
Platelets	150,000– 400,000 per mm ³
Hemoglobin (Hgb)	12 – 16 gm (F); 14 – 18 gm (M).
Hematocrit (Hct)	37 – 47 (F); 40 – 54 (M)

5. SERUM ELECTROLYTES

Sodium	135 – 145 mEq/L
Potassium	3.5 – 5.0 mEq/L
Calcium	8.6–10 mg/dL
Chloride	98 – 107 mEq/L
Magnesium	1.2 – 2.6 mg/dL
Phosphorus	2.7-4.5 mg/dL

6. ACID- BASE BALANCE

Use the ABG Tic-Tac-Toe Method for interpreting. Learn about the technique at: (<https://bit.ly/abgtictactoe>).

pH	7.35 – 7.45
HCO ₃	22 – 26 mEq/L
Pco ₂	35 – 45 mmHg
PaO ₂	80–100 mmHg
SaO ₂	>95

7. CHEMISTRY VALUES

Glucose	70 – 110 mg/dL
BUN	7-22 mg/dL
Serum creatinine	0.6 – 1.35 mg/dL
LDH	100-190 U/L
Protein	6.2 – 8.1 g/dL
Albumin	3.4 – 5.0 g/dL
Bilirubin	<1.0 mg/dL
Total Cholesterol	130 – 200 mg/dL
Triglyceride	40 – 50 mg/dL
Uric acid	3.5 – 7.5 mg/dL
CPK	21-232 U/L

8. URINE TEST NORMAL VALUES

Color	Pale yellow
Odor	Specific aromatic odor, similar to ammonia
Turbidity	Clear
pH	4.5 – 7.8
Specific gravity	1.016 to 1.022
Glucose	<0.5 g/day
Ketones	None
Protein	None
Bilirubin	None
Casts	None to few
Crystals	None
Bacteria	None or <1000/mL
RBC	<3 cells/HPF
WBC	< 4 cells/HPF
Uric Acid	250–750 mg/24 hr

9. NORMAL GLUCOSE VALUES

Glucose, fasting	70 – 110 mg/dL
Glucose, monitoring	60 – 100 mg/dL
Glucose tolerance test, oral	
• Baseline fasting	70 – 110 mg/dL
• 30-min fasting	110 – 170 mg/dL
• 60-min fasting	120 – 170 mg/dL
• 90-min fasting	100 – 140 mg/dL
• 120-min fasting	70 – 120 mg/dL
Glucose, 2-hour postprandial	<140 mg/dL

10. THERAPEUTIC DRUG LEVELS

Acetaminophen (Tylenol)	10-20 mcg/mL
Carbamazepine (Tegretol)	4 – 10 mcg/mL
Digoxin (Lanoxin)	0.5 – 2.0 ng/mL
Gentamycin (Garamycin)	5 – 10 mcg/ml (peak), <2.0 mcg/ml (valley)
Lithium (Eskalith)	0.5 – 1.2 mEq/L
Magnesium sulfate	4 – 7 mg/dL
Phenobarbital (Solfoton)	15 – 40 mcg/mL
Phenytoin (Dilantin)	10 – 20 mcg/dL
Salicylate	100 – 250 mcg/mL
Theophylline (Aminophylline)	10 – 20 mcg/dL
Tobramycin (Tobrex)	5 – 10 mcg/mL (peak), 0.5 – 2.0 mcg/mL (valley)
Valproic Acid (Depakene)	50 – 100 mcg/ml
Vancomycin (Vancocin)	20 – 40 mcg/ml (peak), 5 to 15 mcg/ml (trough)

11. CARDIAC MARKERS

Creatinine kinase (CK)	26 – 174 units/L
• CK-MB	0%-5% of total
• CK-MM	95%-100% of total
• CK-BB	0%
Troponin I	<0.6 ng/mL (> 1.5 ng/mL indicates MI)
Troponin T	> 0.1-0.2 ng/mL indicates MI
Myoglobin	<90 mcg/L; elevation indicates MI
Atrial natriuretic peptides (ANP)	22 – 27 pg/mL
Brain natriuretic peptides (BNP)	< 100 pg/mL

12. ANTICOAGULANT THERAPY

Sodium warfarin (Coumadin) PT	10 – 12 seconds (control). The antidote is Vitamin K.
INR (Coumadin)	0.9 – 1.2
Heparin PTT	30 – 45 seconds (control). The antidote is protamine sulfate.
APTT	3 – 31.9 seconds
Fibrinogen level	203 – 377 mg/dL

17. PREGNANCY CATEGORY OF DRUGS

- **Category A** – No risk in controlled human studies
- **Category B** – No risk in other studies. Examples: Amoxicillin, Cefotaxime.
- **Category C** – Risk not ruled out. Examples: Rifampicin (Rifampin), Theophylline (Theolair).
- **Category D** – Positive evidence of risk. Examples: Phenytoin, Tetracycline.
- **Category X** – Contraindicated in Pregnancy. Examples: Isotretinoin (Accutane), Thalidomide (Immunoprin), etc.
- **Category N** – Not yet classified

18. DRUG SCHEDULES

- **Schedule I** – no currently accepted medical use and for research use only (e.g., heroin, LSD, MDMA).
- **Schedule II** – drugs with high potential for abuse and requires written prescription (e.g., Ritalin, hydromorphone (Dilaudid), meperidine (Demerol), and fentanyl).
- **Schedule III** – requires new prescription after six months or five refills (e.g., codeine, testosterone, ketamine).
- **Schedule IV** – requires new prescription after six months (e.g., Darvon, Xanax, Soma, and Valium).
- **Schedule V** – dispensed as any other prescription or without prescription (e.g., cough preparations, Lomotil, Motofen).

19. MEDICATION CLASSIFICATIONS

- **Antacids** – reduces hydrochloric acid in the stomach.
- **Antianemics** – increases blood cell production.
- **Anticholinergics** – decreases oral secretions.
- **Anticoagulants** – prevents clot formation,
- **Anticonvulsants** – used for management of seizures and/or bipolar disorders.
- **Antidiarrheals** – decreases gastric motility and reduce water in bowel.

- **Antihistamines** – block the release of histamine.
- **Antihypertensives** – lower blood pressure and increases blood flow.
- **Anti-infectives** – used for the treatment of infections,
- **Bronchodilators** – dilates large air passages in asthma or lung diseases (e.g., COPD).
- **Diuretics** – decreases water/sodium from the Loop of Henle.
- **Laxatives** – promotes the passage of stool.
- **Miotics** – constricts the pupils.
- **Mydriatics** – dilates the pupils.
- **Narcotics/analgesics** – relieves moderate to severe pain.

20. RULE OF NINES

- *For calculating Total Body Surface Area (TBSA) for burns:*
- Head and neck: 9%
- Upper limbs: 18% (9% each)
- Anterior torso: 18%
- Posterior torso: 18%
- Legs: 36% (18% each)
- Genitalia: 1%

21. MEDICATIONS

- **Digoxin (Lanoxin)** – Assess pulses for a full minute, if less than 60 bpm hold dose. Check digitalis and potassium levels.
- **Aluminum Hydroxide (Amphojel)** – Treatment of GERD and kidney stones. WOF constipation.
- **Hydroxyzine (Vistaril)** – Treatment of anxiety and itching. WOF dry mouth.
- **Midazolam (Versed)** – given for conscious sedation. Watch out for (WOF) respiratory depression and hypotension.

- **Amiodarone (Cordarone)** – WOF diaphoresis, dyspnea, lethargy. Take missed dose any time in the day or to skip it entirely. Do not take double dose.
- **Warfarin (Coumadin)** – WOF for signs of bleeding, diarrhea, fever, or rash. Stress importance of complying with prescribed dosage and follow-up appointments.
- **Methylphenidate (Ritalin)** – Treatment of ADHD. Assess for heart related side-effects and reported immediately. Child may need a drug holiday because the drug stunts growth.
- **Dopamine** – Treatment of hypotension, shock, and low cardiac output. Monitor ECG for arrhythmias and blood pressure.
- **Rifampicin** – causes red-orange tears and urine.
- **Ethambutol** – causes problems with vision, liver problem.
- **Isoniazid** – can cause peripheral neuritis, take vitamin B6 to counter.

22. DEVELOPMENTAL MILESTONES

- **2 – 3 months:** able to turn head up, and can turn side to side. Makes cooing or gurgling noises and can turn head to sound.
- **4 – 5 months:** grasps, switch and roll over tummy to back. Can babble and can mimic sounds.
- **6 – 7 months:** sits at 6 and waves bye-bye. Can recognize familiar faces and knows if someone is a stranger. Passes things back and forth between hands.
- **8 – 9 months:** stands straight at eight, has favorite toy, plays peek-a-boo.
- **10 – 11 months:** belly to butt.
- **12 – 13 months:** twelve and up, drinks from a cup. Cries when parents leave, uses furniture to cruise.

23. CULTURAL CONSIDERATIONS

- **African Americans** – May believe that illness is caused by supernatural causes and seek advice and remedies from faith healers; they are family oriented; have higher incidence of high blood pressure and obesity; high incidence of lactose intolerance with difficulty digesting milk and milk products.
- **Arab Americans** – May remain silent about health problems such as STIs, substance abuse, and mental illness; a devout Muslim may interpret illness as the will of Allah, a test of faith; may rely on ritual cures or alternative therapies before seeking help from health care provider; after death, the family may want to prepare the body by washing and wrapping the body in unsewn white cloth; postmortem examinations are discouraged unless required by law. May avoid pork and alcohol if Muslim. Islamic patients observe month long fast of Ramadan (begins approximately mid-October); people suffering from chronic illnesses, pregnant women, breast-feeding, or menstruating don't fast. Females avoid eye contact with males; use same-sex family members as interpreters.
- **Asian Americans** – May value ability to endure pain and grief with silent stoicism; typically family oriented; extended family should be involved in care of dying patient; believes in "hot-cold" yin/yang often involved; sodium intake is generally high because of salted and dried foods; may believe prolonged eye contact is rude and an invasion of privacy; may not understand without necessarily understanding; may prefer to maintain a comfortable physical distance between the patient and the health care provider.
- **Latino Americans** – May view illness as a sign of weakness, punishment for evil doing; may consult with a curandero or voodoo priest; family members are typically involved in all aspects of decision making such as terminal illness; may see no reason to submit to mammograms or vaccinations.
- **Native Americans** – May turn to a medicine man to determine the true cause of an illness; may value the ability to endure pain or grief with silent stoicism; diet may be deficient in vitamin D and calcium because many suffer from lactose intolerance or don't drink milk; obesity and diabetes are major health concerns; may divert eyes to the floor when they are praying or paying attention.
- **Western Culture** – May value technology almost exclusively in the struggle to conquer diseases; health is understood to be the absence, minimization, or control of disease process; eating utensils usually consists of knife, fork, and spoon; three daily meals is typical.

24. COMMON DIETS

- **Acute Renal Disease** – protein-restricted, high-calorie, fluid-controlled, sodium and potassium controlled.
- **Addison's disease** – increased sodium, low potassium diet.
- **ADHD and Bipolar** – high-calorie and provide finger foods.
- **Burns** – high protein, high caloric, increase in Vitamin C.
- **Cancer** – high-calorie, high-protein.
- **Celiac Disease** – gluten-free diet (no BROW: barley, rye, oat, and wheat).
- **Chronic Renal Disease** – protein-restricted, low-sodium, fluid-restricted, potassium-restricted, phosphorus-restricted.
- **Cirrhosis (stable)** – normal protein
- **Cirrhosis with hepatic insufficiency** – restrict protein, fluids, and sodium.
- **Constipation** – high-fiber, increased fluids

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- **COPD** – soft, high-calorie, low-carbohydrate, high-fat, small frequent feedings
 - **Cystic Fibrosis** – increase in fluids.
 - **Diarrhea** – liquid, low-fiber, regular, fluid and electrolyte replacement
 - **Gallbladder** diseases – low-fat, calorie-restricted, regular
 - **Gastritis** – low-fiber, bland diet
 - **Hepatitis** – regular, high-calorie, high-protein
 - **Hyperlipidemias** – fat-controlled, calorie-restricted
 - **Hypertension**, heart failure, CAD – low-sodium, calorie-restricted, fat-controlled
 - **Kidney Stones** – increased fluid intake, calcium-controlled, low-oxalate
 - **Nephrotic Syndrome** – sodium-restricted, high-calorie, high-protein, potassium-restricted.
 - **Obesity, overweight** – calorie-restricted, high-fiber

- **Pancreatitis** – low-fat, regular, small frequent feedings; tube feeding or total parenteral nutrition.
- **Peptic ulcer** – bland diet
- **Pernicious Anemia** – increase Vitamin B12 (Cobalamin), found in high amounts on shellfish, beef liver, and fish.
- **Sickle Cell Anemia** – increase fluids to maintain hydration since sickling increases when patients become dehydrated.
- **Stroke** – mechanical soft, regular, or tube-feeding.
- **Underweight** – high-calorie, high protein
- **Vomiting** – fluid and electrolyte replacement

13. UNIT CONVERSIONS

1 teaspoon (t)	5 ml
1 tablespoon (T)	3 t (15 ml)
1 oz	30 ml
1 cup	8 oz
1 quart	2 pints
1 pint	2 cups
1 grain (gr)	60 mg
1 gram (g)	1,000 mg
1 kilogram (kg)	2.2 lbs
1 lb	16 oz
Convert C to F	multiply by 1.8 then add 32
Convert F to C:	subtract 32 then divide by 1.8

14. MATERNITY NORMAL VALUES

- **Fetal Heart Rate:** 120 – 160 bpm
- **Variability:** 6 – 10 bpm
- **Amniotic fluid:** 500 – 1200 ml
- **Contractions:** 2 – 5 minutes apart with duration of < 90 seconds and intensity of <100 mmHg.
- **AVA:** The umbilical cord has two arteries and one vein.

15. APGAR SCORING

- **A**ppearance, **P**ulses, **G**rimace, **A**ctivity, **R**eflex Irritability.
- Done at 1 and 5 minutes with a score of 0 for absent, 1 for decreased, and 2 for strongly positive.
- Scores 7 and above are generally normal, 4 to 6 fairly low, and 3 and below are generally regarded as critically low.

16. EPIDURAL ANESTHESIA: STOP

- STOP is a treatment for maternal hypotension after an epidural anesthesia.
- Stop infusion of Pitocin.
- Turn the client on her left side.
- Oxygen therapy.
- Push IV fluids, if hypovolemia is present.

25. POSITIONING CLIENTS

- **Asthma** – Orthopneic position where patient is sitting up and bent forward with arms supported on a table or chair arms.
- **Post Bronchoscopy** – flat on bed with head hyperextended.
- **Cerebral Aneurysm** – high Fowler's.
- **Hemorrhagic Stroke** – HOB elevated 30 degrees to reduce ICP and facilitate venous drainage.
- **Ischemic Stroke** – HOB flat.
- **Cardiac Catheterization** – keep site extended.
- **Epistaxis** – lean forward.
- **Above Knee Amputation** – elevate for first 24 hours on pillow, position on prone daily for hip extension.
- **Below Knee Amputation** – foot of bed elevated for first 24 hours, position prone daily for hip extension.
- **Tube feeding for patients with decreased LOC** – position patient on right side to promote emptying of the stomach with HOB elevated to prevent aspiration.
- **Air/Pulmonary embolism** – turn patient to left side and lower HOB.
- **Postural Drainage** – Lung segment to be drained should be in the uppermost position to allow gravity to work.
- **Post Lumbar puncture** – patient should lie flat in supine to prevent headache and leaking of CSF.
- **Continuous Bladder Irrigation (CBI)** – catheter should be taped to thigh so legs should be kept straight.
- **After myringotomy** – position on the side of affected ear after surgery (allows drainage of secretion).
- **Post cataract surgery** – patient will sleep on unaffected side with a night shield for 1-4 weeks.
- **Detached retina** – area of detachment should be in the dependent position.

- **Post thyroidectomy** – low or semi-Fowlers, support head, neck and shoulders.
- **Thoracentesis** – sitting on the side of the bed and leaning over the table (during procedure); affected side up (after procedure).
- **Spina Bifida** – position infant on prone so that sac does not rupture.
- **Buck's Traction** – elevate foot of bed for counter-traction.
- **Post Total Hip Replacement** – don't sleep on operated side, don't flex hip more than 45-60 degrees, don't elevate HOB more than 45 degrees. Maintain hip abduction by separating thighs with pillows.
- **Prolapsed cord** – knee-chest position or Trendelenburg.
- **Cleft-lip** – position on back or in infant seat to prevent trauma to the suture line. While feeding, hold in upright position.
- **Cleft-palate** – prone.
- **Hemorrhoidectomy** – assist to lateral position.
- **Hiatal Hernia** – upright position.
- **Preventing Dumping Syndrome** – eat in reclining position, lie down after meals for 20-30 minutes (also restrict fluids during meals, low fiber diet, and small frequent meals).
- **Enema Administration** – position patient in left-side lying (Sim's position) with knees flexed.
- **Post supratentorial surgery (incision behind hairline)** – elevate HOB 30-45 degrees.
- **Post infratentorial surgery (incision at nape of neck)** – position patient flat and lateral on either side.
- **Increased ICP** – high Fowler's.
- **Laminectomy** – back as straight as possible; log roll to move and sand bag on sides.

- **Spinal Cord Injury** – immobilize on spine board, with head in neutral position. Immobilize head with padded C-collar, maintain traction and alignment of head manually. Log roll client and do not allow client to twist or bend.
- **Liver Biopsy** – right side lying with pillow or small towel under puncture site for at least 3 hours.
- **Paracentesis** – flat on bed or sitting.
- **Intestinal Tubes** – place patient on right side to facilitate passage into duodenum.
- **Nasogastric Tubes** – elevate HOB 30 degrees to prevent aspiration. Maintain elevation for continuous feeding or 1 hour after intermittent feedings.
- **Rectal Exam** – knee-chest position, Sim's, or dorsal recumbent.
- **During internal radiation** – patient should be on bed rest while implant is in place.
- **Autonomic Dysreflexia** – place client in sitting position (elevate HOB) first before any other implementation.
- **Shock** – bed rest with extremities elevated 20 degrees, knees straight, head slightly elevated (modified Trendelenburg).
- **Head Injury** – elevate HOB 30 degrees to decrease intracranial pressure.
- **Peritoneal Dialysis when outflow is inadequate** – turn patient side to side before checking for kinks in the tubing.
- **Myelogram Water-based dye** – semi Fowler's for at least 8 hours.
- **Myelogram Oil-based dye** – flat on bed for at least 6-8 hours to prevent leakage of CSF.
- **Myelogram Air dye** – Trendelenburg

26. COMMON SIGNS AND SYMPTOMS

- **Pulmonary Tuberculosis (PTB)** – low-grade afternoon fever.
- **Pneumonia** – rust-colored sputum.
- **Asthma** – wheezing on expiration.
- **Emphysema** – barrel chest.
- **Kawasaki Syndrome** – strawberry tongue.
- **Pernicious Anemia** – red beefy tongue.
- **Down syndrome** – protruding tongue.
- **Cholera** – rice-watery stool and washer woman's hands (wrinkled hands from dehydration).
- **Malaria** – stepladder like fever with chills.
- **Typhoid** – rose spots in the abdomen.
- **Dengue** – fever, rash, and headache. Positive Herman's sign.
- **Diphtheria** – pseudomembrane formation.
- **Measles** – Koplik's spots (clustered white lesions on buccal mucosa).
- **Systemic Lupus Erythematosus** – butterfly rash.
- **Leprosy** – leonine facies (thickened folded facial skin).
- **Bulimia** – chipmunk facies (parotid gland swelling).
- **Appendicitis** – rebound tenderness at McBurney's point. Rovsing's sign (palpation of LLQ elicits pain in RLQ). Psoas sign (pain from flexing the thigh to the hip).
- **Meningitis** – Kernig's sign (stiffness of hamstrings causing inability to straighten the leg when the hip is flexed to 90 degrees), Brudzinski's sign (forced flexion of the neck elicits a reflex flexion of the hips).

- **Tetany** – hypocalcemia, [+] Trousseau's sign; Chvostek sign.
- **Tetanus** – Risus sardonius or rictus grin.
- **Pancreatitis** – Cullen's sign (ecchymosis of the umbilicus), Grey Turner's sign (bruising of the flank).
- **Pyloric Stenosis** – olive like mass.
- **Patent Ductus Arteriosus** – washing machine-like murmur.
- **Addison's disease** – bronze-like skin pigmentation.
- **Cushing's syndrome** – moon face appearance and buffalo hump.
- **Grave's Disease (Hyperthyroidism)** – Exophthalmos (bulging of the eye out of the orbit).
- **Intussusception** – Sausage-shaped mass.
- **Multiple Sclerosis** – Charcot's Triad: nystagmus, intention tremor, and dysarthria.
- **Myasthenia Gravis** – descending muscle weakness, ptosis (drooping of eyelids).
- **Guillain-Barre Syndrome** – ascending muscles weakness.
- **Deep vein thrombosis (DVT)** – Homan's Sign.
- **Angina** – crushing, stabbing pain relieved by NTG.
- **Myocardial Infarction (MI)** – crushing, stabbing pain radiating to left shoulder, neck, and arms. Unrelieved by NTG.
- **Parkinson's disease** – pill-rolling tremors.
- **Cytomegalovirus (CMV) infection** – Owl's eye appearance of cells (huge nucleus in cells).

- **Glaucoma** – tunnel vision.
- **Retinal Detachment** – flashes of light, shadow with curtain across vision.
- **Basilar Skull Fracture** – Raccoon eyes (periorbital ecchymosis) and Battle's sign (mastoid ecchymosis).
- **Buerger's Disease** – intermittent claudication (pain at buttocks or legs from poor circulation resulting in impaired walking).
- **Diabetic Ketoacidosis** – acetone breathe.
- **Pregnancy Induced Hypertension (PIH)** – proteinuria, hypertension, edema.
- **Diabetes Mellitus** – polydipsia, polyphagia, polyuria.
- **Gastroesophageal Reflux Disease (GERD)** – heartburn.
- **Hirschsprung's Disease (Toxic Megacolon)** – ribbon-like stool.
- **Herpes Simplex Type II** – painful vesicles on genitalia
- **Genital Warts** – warts 1-2 mm in diameter.
- **Syphilis** – painless chancres.
- **Chancroid** – painful chancres.
- **Gonorrhea** – green, creamy discharges and painful urination.
- **Chlamydia** – milky discharge and painful urination.
- **Candidiasis** – white cheesy odorless vaginal discharges.
- **Trichomoniasis** – yellow, itchy, frothy, and foul-smelling vaginal discharges

27. MISCELLANEOUS TIPS

- **Delegate** sterile skills (e.g., dressing change) to the RN or LPN.
- Where non-skilled care is required, delegate the stable client to the nursing assistant.
- Assign the most critical client to the RN.
- Clients who are being discharged should have final assessments done by the RN.
- The Licensed Practical Nurse (LPN) can monitor clients with IV therapy, insert urinary catheters, feeding tubes, and apply restraints.
- Assessment, teaching, medication administration, evaluation, unstable patients cannot be delegated to an unlicensed assistive personnel.
- **Weight** is the best indicator of dehydration.

- Para is the number of pregnancies that reached viability, regardless of whether the fetus was delivered alive or stillborn. A fetus is considered viable at 20 weeks' gestation.
- **Lochia rubra** is the vaginal discharge of almost pure blood that occurs during the first few days after childbirth.
- **Lochia serosa** is the serous vaginal discharge that occurs 4 to 7 days after childbirth.
- **Lochia alba** is the vaginal discharge of decreased blood and increased leukocytes that's the final stage of lochia. It occurs 7 to 10 days after childbirth.
- In the event of fire, the acronym most often used is **RACE**. (R) Remove the patient. (A) Activate the alarm. (C) Attempt to contain the fire by closing the door. (E) Extinguish the fire if it can be done safely.

- Before signing an **informed consent form**, the patient should know whether other treatment options are available and should understand what will occur during the preoperative, intraoperative, and postoperative phases; the risks involved; and the possible complications. The patient should also have a general idea of the time required from surgery to recovery. In addition, he should have an opportunity to ask questions.
- The first nursing intervention in a quadriplegic client who is experiencing **autonomic dysreflexia** is to elevate his head as high as possible.
- Usually, patients who have the same infection and are in strict isolation can share a room.
- **Veracity** is truth and is an essential component of a therapeutic relationship between a health care provider and his patient.

- When patient is in distress, administration of medication is rarely the best choice.
- Always check for allergies before administering antibiotics.
- Neutropenic patients should not receive vaccines, fresh fruits, or flowers.
- Nitroglycerine sublingual is administered up to three times with intervals of five minutes.
- Morphine is contraindicated in pancreatitis because it causes spasms of the Sphincter of Oddi. Demerol should be given.
- Never give potassium (K+) in IV push.
- Infants born to an HIV-positive mother should receive all immunizations of schedule.
- Gravida is the number of pregnancies a woman has had, regardless of outcome.

- **Beneficence** is the duty to do no harm and the duty to do good. There's an obligation in patient care to do no harm and an equal obligation to assist the patient.
- **Nonmaleficence** is the duty to do no harm.
- **Tyramine-rich food**, such as aged cheese, chicken liver, avocados, bananas, meat tenderizer, salami, bologna, Chianti wine, and beer may cause severe hypertension in a patient who takes a monoamine oxidase inhibitor.
- **Projection** is the unconscious assigning of a thought, feeling, or action to someone or something else.
- **Sublimation** is the channeling of unacceptable impulses into socially acceptable behavior.
- **Repression** is an unconscious defense mechanism whereby unacceptable or painful thoughts, impulses, memories, or feelings are pushed from the consciousness or forgotten.

- People with **obsessive-compulsive disorder** realize that their behavior is unreasonable, but are powerless to control it.
- A significant toxic risk associated with **clozapine (Clozaril)** administration is **blood dyscrasia**.
- Adverse effects of **haloperidol (Haldol)** administration include drowsiness; insomnia; weakness; headache; and extrapyramidal symptoms, such as akathisia, tardive dyskinesia, and dystonia.
- **Hypervigilance** and déjà vu are signs of posttraumatic stress disorder (PTSD)

